



(<https://learningcenter.hfsa.org/>).



My Courses (<https://learningcenter.hfsa.org/Users/Home.aspx>)
/ **2026 AHFTC Board Certification Review - OnDemand**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=4f%2fOexzNsorKAF1lCFotvA%3d%3d>)
/ **2026 AHFTC Board Certification Question Bank**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=YSpCYks4EUa7JCy700Rkkw%3d%3d>)
/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 103 of 117



A 38 year old presents to the emergency room with four days of shortness of breath, chest discomfort, and nausea. Upon arrival, heart rate is 115 beats per minute and blood pressure is 95/65 mm Hg. On physical examination, jugular venous pressure is 15 cm H₂O, lungs are clear to auscultation and cardiac exam reveals tachycardia and an S3 gallop. Extremities are cool. Beside echocardiography reveals severely reduced left ventricular systolic function. 12-lead electrocardiography reveals sinus tachycardia with deep T wave inversions throughout the precordium and poor R-wave progression. ON telemetry, frequent episodes of non-sustained ventricular tachycardia are seen.

Which is the next best diagnostic test?

- A. Cardiac magnetic resonance imaging
- ✓ B. Right heart catheterization with endomyocardial biopsy
- C. Pharmacologic myocardial perfusion stress imaging
- D. Coronary angiogram
- E. Serum viral serologies

Commentary

References

Testing Point: The Board-certified AHFTC specialist should recognize the indications for an endomyocardial biopsy in the setting of acute onset heart failure with hemodynamic consequences.

Answer: B. Right heart catheterization with endomyocardial biopsy.

Rationale: According to American College of Cardiology/American Heart Association guidelines, endomyocardial biopsy is a Class I recommendation for patients with new-onset heart failure of less than 2 weeks' duration associated with a normal-sized or dilated left ventricle and hemodynamic compromise.

Answer A: Cardiac magnetic resonance imaging is not unreasonable but may not provide important hemodynamic information in a patient nearing cardiogenic shock and is therefore a less preferable choice. CMR may demonstrate late gadolinium enhancement patterns consistent with myocarditis, but biopsy provides a more definitive diagnosis, including characterization of the inflammatory infiltrate.

Answers C and D: Pharmacologic myocardial perfusion imaging and coronary angiography are not the best next diagnostic tests. Although the patient presents with chest discomfort and ECG abnormalities, the clinical picture of acute decompensated heart failure with ventricular arrhythmias and hemodynamic compromise is more concerning for fulminant myocarditis, for which tissue diagnosis is urgently needed.

Answer E: Serum viral serologies are low yield and rarely alter management in suspected myocarditis. They cannot confirm myocardial involvement or provide histopathologic diagnosis. In hemodynamically unstable patients, endomyocardial biopsy is preferred because it provides definitive diagnosis and may guide urgent treatment.

Powered by Oasis.



(<https://www.oasis-lms.com/>).